



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : بسمه حافظ حسيني محمد

المحافظة : القاهرة الحي / المدينة : حلوان

العنوان : 16 ش المنفلوطي - ت المعلمين - ح حلوان

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 28606151404201 رقم التليفون : 01015971510

العمر (تاريخ الميلاد) : 15-06-1986 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / أ.د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **بسمه حافظ حسيني محمد**
National ID: **28606151404201**



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



بسمه
 حافظ حسيني محمد الصليبي
 ١٦ ش النفلوطيت المحمدين - ح جنوان
 حنوان - القاهرة

١٩٨٧/٠٣/٠٥

٢٨٦٠٦١٥١٤٠٤٢٠١

KP8447068

١٥٨٧

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٠١٢٢١١٦٦٢٠٩

مقطعي

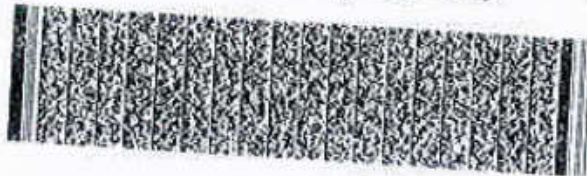
تدريس

٧٠ مع ذري الوزن



٢٠١٤/٠٧ ٢٨٦٠٦١٥١٤٠٤٢٠١
 حاصلة على بكالوريوس تربية نوعية

متزوجة مسلمة
 وليد فرج عبدالقادر عمران
 البطاقة سنوية حتى ٢٠٢١/٠٧/٠١





MINISTRY OF HEALTH

قسم الباثولوجي

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: بسمه حافظ حسيني محمد

Age: 46 years

Sex: female

Request type: ميانبره

Sub request type:

Hospital no:

Pathology no.: 733-2026

Department: صحنه مرأه

Date of result: 8/8/2026

Referred by: د/مروه رمزي

Date of biopsy: 29/7/2026

Nature of specimen: tru-cut biopsy

I.D. Number: 28606151404201

Clinical diagnosis: Right breast mass

Gross pathology:

Multiple fragmented grayish white tissue cores up to 0.5x0.4cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: Ecadherine- ER- PR- Her2- Ki67

Microscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor forms nests, tubule and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing marked anaplasia and mild tendency to form tubules. There is moderate number of mitotic figures. Ductal carcinoma insitu component (comedo subtype) could be seen.

Immunohistochemical Result

Ecadherine: positive in tumor cells

ER: Weak nuclear staining in 20% of tumor cells (Allred Score 4/8).

PR: Weak nuclear staining in 30% of tumor cells (Allred Score 4/8).

Her2: Strong positive complete membranous staining in >10% of tumor cells (score 3+) positive.

Ki67: Positive nuclear staining of 70% of tumor cells.

Diagnosis: Right breast mass, tru-cut biopsy:

Invasive duct carcinoma (NST), Grade III.

Signature:

Dr. Mona Mohamed.M.D

Pathology Lab. Manager

BASMA REFAAT M.D

Patient ID: 97476	Patient Name English: Basma Hafez Husainy
Age: 38 YEARS	2026/07/21 01:32:52 PM :Report Date

Women's Health Outreach Program

Bilateral full field Digital Mammography

Clinical summary: Right 2 palpable lumps

Technique

Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Right breast

- Dense breast lower the sensitivity of the study (ACR D)
- UOQ & upper central 2 irregular spiculated dense masses with distortion with associated grouped calcifications
- **Ultrasound** : 10:00 zone A & C 2 irregular hypoechoic masses with distortion measures 1.9x0.8 & 2.3x1.4cm respectively (correspond to patient complain)
- 10-12:00 area of altered parenchyma and distortion
- Normal skin and nipple/areolar complex.
- Pathologically enlarged level I axillary nodes, more than 3 all are discrete, the largest show thickened cortex & indenting hilum measure 2.5x1.3cm

Left breast

- Dense breast lower the sensitivity of the study (ACR D)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADSS): UOQ 2 malignant mass lesion as described for US guided biopsy & contrast based study.

Left breast (BI-RADS 2): Benign

Dr. Marwa Ramzy, MD, FRCR,

 		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية		استشارة مختصة
MDT FORM		08/30/2026		
Name: <u>باسم حاتم حسان محمد</u>		MRN: 13223		
Age: 40 Y		ID: 28606151404201		
Menopausal Sta: PRE		Phone:* 01015971510		
NRS SCORE:		Marital Status:		
BMI:		Family History: -VE		
Gender:		Reason Of MDT:		
CO Morbidity: M.FREE		NEW CASE		
Surgical History:* LT KNEE LATERAL MENISCUS TEAR REPAIR 2 YEARS AGO		ECOG Performance Status		
		☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4		

Clinical Data:

BX: RT. IDC G3
ER: 4/8 **PR:** 4/8 **HER2:** +3 **KI67:** 70%

History:**Sonomammography:**

SM>> ACR D >> RT. UOQ & UPPER CENTRAL 2 MASSES WITH DISTORTION & GROUPED CALCIFICATIONS AT 10 OCLOCK ZONE A & C WITH DISTORTION 1.9*0.8 CM & 2.3*1.4 CM
 10-12 OCLOCK AREA OF PARENCHYMAL DISTORTION
 PATHOLOGICAL ENLARGED LN MORE THAN 3 WITH THICKENED CORTEX
 BIRADS 5

Biomarker/Molecular

CESM >> RT. UOQ & LESSER EXTENT UTQ ENHANCING MASSES LARGEST 1.9*1.8 CM
 THE OVERALL AREA 9*10*8*5 CM
 WITH MULTIPLE PATHOLOGICAL AXILLARY LN MORE THAN 3

Mets Workup:

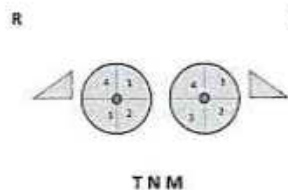
PET CT >> FREE

Recommendations:

FOR BRACKETING THEN NACT THEN RT BCS & ALND

MDT Outcome & Plan

FOR BRACKETING
 THEN Neoadj TCHP
 THEN assess for possibility RT BCS & ALND



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

CT2N1M0

Pathological Staging:**SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حنان عزت	د. أحمد نجيب		

مستشفى
رونترو
بجدة



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
بجدة

08/30/2026

MDT FORM

Name:	د. محمد بن عبد الله بن محمد	MRN:	13223
Age:	40 Y	ID:	28606151404201
Menopausal Sta	PRE	Phone:*	01015971510
NRS SCORE:		Marital Status:	
BMI:		Family History:	-VE
Gender		Reason Of MDT:	NEW CASE
CO Morbidity:	M.FREE	ECOG Performance Status	0 10 20 30 4
Surgical History: * LT KNEE LATERAL MENISCUS TEAR REPAIR 2 YEARS AGO			

Clinical Data:

BX: RT. IDC G3

ER: 4/8 PR: 4/8 HER2: +3 KI67: 70%

History:

Sonomammography:

SM>> ACR D >> RT. UOQ & UPPER CENTRAL. 2 MASSES WITH DISTORTION & GROUPED CALCIFICATIONS AT 10 O'CLOCK ZONE A& C WITH DISTORTION 1.9*0.8 CM & 2.3*1.4 CM
10-12 O'CLOCK AREA OF PARENCHYMAL DISTORTION
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BIRADS 5
Biomarker/Molecular
CESM >> RT.UOQ& LESSER EXTENT UIQ ENHANCING MASSES LARGEST 1.9*1.8 CM
THE OVERALL AREA 9*10*8*5 CM
WITH MULTIPLE PATHOLOGICAL AXILARY LN MORE THAN 3

Metas Workup:

PET CT >> FREE

Recommendations:

FOR BRACKETING THEN NACT THEN RT BCS & ALND

MDT Outcome & Plan

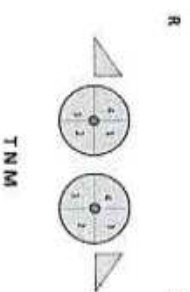
FOR BRACKETING

THEN Neoadj TCHP

THEN assess for possibility RT BCS & ALND

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد بن عبد الله بن محمد	د. أحمد بن محمد	د. محمد بن عبد الله بن محمد	د. محمد بن عبد الله بن محمد



Clinical Staging:

CT2NM0

Pathological Staging:

Visit Number: 10382605896

Client Name:

نقابة المهندسين - مشركى مشروع العلاج اشعة
(فقط)

Patient Name: Umar Isah dani

Referred by Prof:

Age / Sex: 40 year / Female

Exam Date: 16 Aug 2026 00:00

Patient ID: 2141986325

Printed: 8/17/2026 11:41:43 AM

PT

WHOLE BODY PET/CT EXAMINATION

History and Clinical Data: -

Patient data:-

Female patient, 40 years old, Weight: 69 Kg, Blood Glucose level: 101 mg/dl.

Radiopharmaceutical:-

F-18-FDG 7.3 mCi injected I.V.

Clinical background:-

Recently diagnosed right breast carcinoma.

Purpose of the study:-

Assessment and metastatic work up.

TECHNIQUE:-

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~1.5.
- Liver showed max. SUV ~2.1.

REPORT:-

Review of the PET, CT and the fused images revealed:

Loco-regional (breasts and axillae):

Right side:

The right breast is the seat of two metabolically active FDG avid soft tissue lesions at UOQ & outer center region; measuring roughly up to 2 cm with max. SUV~10.5.

Few metabolically active FDG avid right axillary LNs are seen measuring up to 2 cm with max. SUV~15.8.

Left side:

No FDG avid left breast masses.



Visit Number: 10382605896

Client Name: نقابة المهندسين - مشيركي مشروع العلاج الشعاع (فقط)

Patient Name: نبسة حناظ حسن

Referred by Prof: _____

Age / Sex: 40 year / Female

Exam Date: 16 Aug 2026 00:00

Patient ID: 2141986325

Printed: 8/17/2026 11:41:43 AM

PT

No FDG avid regional pathological LNs.

No FDG avid distant metastases.

1) The rest of the body:

Average sized liver showing normal FDG uptake with no metabolically active FDG avid focal lesions.

Average sized spleen showing normal FDG uptake with no metabolically active FDG avid focal lesions.

Clear both lungs apart from fine apical reticulations. No FDG avid pulmonary masses or nodules.

Mildly increased bone marrow FDG uptake; likely marrow hyperplasia. No FDG avid lytic/sclerotic osseous lesions.

No FDG avid adrenal or pancreatic masses.

No ascites or pleural effusion.

Other Findings:

An IUD is seen within the uterus.

Right ovarian active cyst (likely functional).

1) The rest of PET / CT study appears normal.

Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, and urinary tract.

CONCLUSION:

- PET-CT study for reported case of breast carcinoma showing:
 - ✓ Metabolically active (FDG avid) right breast bi-focal neoplastic soft tissue lesions; associated with FDG avid regional metastatic axillary LNs.
 - ✓ No FDG avid distant metastases.

Dr. Sherif Fifi.

Prof. Ahmed Wafaie MD, FRCR

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

09/06/2026

1

MDT FORM

Name:	Suresh Kumar	MRN:	13223
Age:	40 Y	ID:	28606151404201
Menopausal Sta		Phone:*	01015971510
NRS SCORE:		Marital Status:	Married
BMI:		Family History:	-ve
Gender		Reason Of MDT:	
CO Morbidity:	M, FREE	REMDT	
Surgical History:*	LT KNEE LATERAL MENISCUS TEAR REPAIR 2 YEARS AGO	ECOG Performance Status	0 0 1 0 2 0 3 0 4

Clinical Data:

3

2

BK: **IDC G 111**
ER: **PR:** **HER2:** **K167:** 70%



T N 64

History:

Sonoma Geography:

Session 10: Imaging of the Liver
SM -> ACR D -> RT. UOQ & UPPER CENTRAL 2 MASSES WITH DISTORTION & GROUPED CALCIFICATIONS AT 10 O'CLOCK ZONE A& C WITH DISTORTION 1.9*0.8 CM & 2.3*1.4 CM
10-12 O'CLOCK AREA OF PARENCHYMAL DISTORTION
PATHOLOGICAL ENLARGED LN MORE THAN 3 WITH THICKENED CORTEX
BRAD5 5
Biomarker/Molecular
ESM -> RTUOQ8 LESSER EXTENT UOQ ENHANCING MASSES LARGEST 1.9*1.8 CM
THE OVERALL AREA 9*10*8*5 CM
WITH MULTIPLE PATHOLOGICAL AXILLARY LN MORE THAN 3
Mets Workup:

Clinical Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Pathological Staging:

Pathological Staging:

Recommendations:

For BRACKETING clips :intervention radiology sees that process reaching multiple involving whole UOQ extending to UQ mass and non mass enhancement

MDT Outcome & Plan

for NACT + postmenstrual
TCHP ~~6~~bc
THEN NIPPLE SAVING MASTECTOMY + ALND AFTER PATIENT
CONSENSING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
2	1		

Appt. Name: Basma Chafiz Hussein	Pat. ID: 14546170	مركز الأشعة الطبية
Procedure Name: Echocardiography	Date: 01/09/2026	
Ref. Doctor: Bedon Roshth		

Echocardiography and Doppler Report

LVEDD	4.9	3.5 - 5.7	LA	3.2	3.2 - 4
LVESD	2.9		AO	2.7	
SWT	0.7	0.6 - 1.1	EF	70%	
PWT	0.7	0.6 - 1.1	FS %		

Comment:

- ❖ **Left Ventricle:**
 - Normal internal dimensions with normal wall thickness and good global contractility
 - No evidence of segmental wall motion abnormalities at rest.
 - Doppler mitral inflow: Normal pattern.
- ❖ **Left Atrium:** Normal dimensions.
- ❖ **Mitral Valve:** Normal leaflets thickness with good excursion and normal mobility. Normal subvalvular apparatus. No calcification. Trivial regurgitation, and no significant diastolic gradient.
- ❖ **Aortic Valve:** Trileaflet valve with normal thickness and good excursion. No regurgitation, and no significant systolic gradient.
- ❖ **Right Side Chambers:** Normal right ventricular dimensions with normal systolic function (TAPSE = 24 mm). Normal right atrial dimensions.
- ❖ **Tricuspid Valve:** Normal leaflets thickness with good excursion and normal mobility. Trivial regurgitation (EPASP = 21 mmHg).
- ❖ **Pulmonary Valve:** Normal leaflets thickness with good excursion and normal mobility. No regurgitation.
- ❖ No intracardiac masses or thrombi could be detected by TTE.
- ❖ Intact both interatrial and interventricular septa.
- ❖ No pericardial effusion.

Conclusion:

Normal transthoracic echocardiographic study.

Dr. Wafaa Hamed, MSc

Hotline



19989

www.technoscan.com.eg

SANUSING 14545170 (29-08-1986)
Hussien, Basma Hafez
Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%

1 D 2.74 cm
2 D 3.25 cm



01-09-2026
12:07:12 AM
Tb 0.5 Tib 0.8 M 1.4 Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%



SANUSING 14545170 (29-08-1986)
Hussien, Basma Hafez
Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%

1 IVSd 0.78 cm
2 LVIDd 3.25 cm
LVEDV (M) (Teichholz) 11.382 ml
3 LVEDVd 0.77 cm
LV Mass (G) 122.40 g
IVSd (LV/PWd) (M) 1.06
RWT (M) 0.79
4 LVIDs 2.95 cm
LV % FS (M) 40.54 %
LVESV (M) (Teichholz) 33.48 ml
LV SV (M) (Teichholz) 80.95 ml
LV EE (M) (Teichholz) 70.74 %

SANUSING 14545170 (29-08-1986)
Hussien, Basma Hafez
Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%

1 TAPSE 24.05 mm



01-09-2026
12:07:07 AM
Tb 0.5 Tib 0.8 M 1.4 Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%



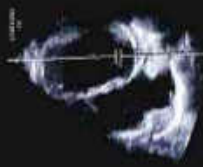
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Hussien, Basma Hafez
Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%



01-09-2026
12:10:12 AM
Tb 0.8 Tib 2.4 M 0.35 Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%

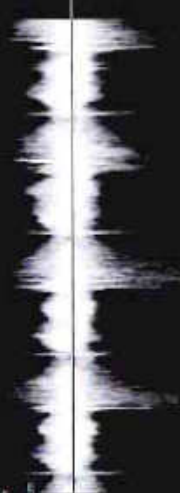


SANUSING 14545170 (29-08-1986)
Hussien, Basma Hafez
Adult Echo: PAT-54, 15.0cm, 30Hz → E

1201
Gen 35
On 110
PA 3 100%
[M] 35
Gen 35
On 110
PA 3 100%



1 Vel -204.29 cm/s
PGmax 16.09 mmHg



جمهورية مصر العربية
بطاقة تحقيق الشخصية



بسمه

حافظ حسيني محمد الصيني

١٦ ش المنفلوطي - ت المعلمين - ح حلوان

حلوان - القاهرة

٢٩٨٦/٠



٢٨٦.٦١٥١٤.٤٢.١

KP8447068

Ministry of
Interior



٢٠٢٤/٠٧ ٢٨٦٠٦١٥١٤٠٤٢٠١

حاصلة على بكالوريوس تربية نوعية

أنثى مسلمة متزوجة

وليد فرج عبدالقادر عمران

البطاقة سارية حتى ٢٠٣١/٠٧/٠١



Patient ID: 97476	Patient Name English: Basma Hafez Husainy
Age: 38 YEARS	2026/07/21 01:32:52 PM :Report Date

Women's Health Outreach Program
Bilateral full field Digital Mammography

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- o **Ultrasound** demonstrated no solid or cystic lesions.
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Impression:

Right breast (BIRADS5): UOQ 2 malignant mass lesion as described for US guided biopsy & contrast based study.

Left breast (BI-RADS 2) : Benign

Dr. Marwa Ramzy, MD, FRCR



الجمهورية العربية السورية
الوزارة
الطب
الأمراض المعدية

الهيئة العامة للمستشفيات والمعاهد التخصصية
لمعهد القومى للكبد والجهاز الهضمي
الأمراض المعدية

قسم الباثولوجى

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: بسمه حافظ حسيني محمد
Request type: مبادره
Hospital no:
Department: صحه مزاج
Referred by: د/مروه رمزي
Nature of specimen: tru-cut biopsy
Clinical diagnosis: Right breast mass

40
Age: 46 years
Sex: female
Sub request type:
Pathology no.: 733-2026
Date of result: 8/8/2026
Date of biopsy: 29/7/2026
I.D. Number: 28606151404201

Gross pathology:

Multiple fragmented grayish white tissue cores up to 0.5x0.4cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: Ecadherine- ER- PR- Her2- Ki67

Micoscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor forms nests, tubule and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing marked anaplasia and mild tendency to form tubules. There is moderate number of mitotic figures. Ductal carcinoma in situ component (comedo subtype) could be seen.

Immunohistochemical Result

Ecadherine: positive in tumor cells

ER: Weak nuclear staining in 20% of tumor cells (Allred Score 4/8).

PR: Weak nuclear staining in 30% of tumor cells (Allred Score 4/8).

Her2: Strong positive complete membranous staining in >10% of tumor cells (score 3+) positive.

Ki67: Positive nuclear staining of 70% of tumor cells.

Diagnosis: Right breast mass, tru-cut biopsy:

Invasive duct carcinoma (NST), Grade III.

Signature:

Dr.Mona Mohamed.M.D

MD

Pathology Lab. Manager

SASMA REFAAT M.D

Visit Number:	10382605896	Client Name:	نقابة المهندسين - مشرفى مشروع العلاج اشعة (فقط)
Patient Name:	بسمه حافظ حسين	Referred by Prof:	
Age / Sex:	40 year / Female	Exam Date:	16 Aug 2026 00:00
Patient ID:	2141986325	Printed:	8/17/2026 11:41:43 AM

PT

WHOLE BODY PET/CT EXAMINATION**History and Clinical Data: -****Patient data: -**

Female patient, 40 years old, Weight: 69 Kg, Blood Glucose level: 101 mg/dl.

Radiopharmaceutical: -

F-18-FDG 7.3 mCi injected I.V.

Clinical background: -

Recently diagnosed right breast carcinoma.

Purpose of the study: -

Assessment and metastatic work up.

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~1.5.
- Liver showed max. SUV ~2.1.

REPORT: -**Review of the PET, CT and the fused images revealed:****Loco-regional (breasts and axillae):****Right side:**

The right breast is the seat of two metabolically active FDG avid soft tissue lesions at UOQ & outer center region; measuring roughly up to 2 cm with max. SUV~10.5.

Few metabolically active FDG avid right axillary LNs are seen measuring up to 2 cm with max. SUV~15.8.

Left side:

No FDG avid left breast masses.

Visit Number: 10382605896

Client Name:

تقاربة المهندسين - مشرقي مشروع العلاج الشعاعية (فقط)

Patient Name:

بسمه حافظ حسين

Referred by Prof:

Age / Sex:

40 year / Female

Exam Date: 16 Aug 2026 00:00

Patient ID:

2141986325

Printed: 8/17/2026 11:41:43 AM

PT

No FDG avid regional pathological LNs.

No FDG avid distant metastases.**1) The rest of the body:**

Average sized liver showing normal FDG uptake with no metabolically active FDG avid focal lesions.

Average sized spleen showing normal FDG uptake with no metabolically active FDG avid focal lesions.

Clear both lungs apart from fine apical reticulations. No FDG avid pulmonary masses or nodules. Mildly increased bone marrow FDG uptake; likely marrow hyperplasia. No FDG avid lytic/sclerotic osseous lesions.

No FDG avid adrenal or pancreatic masses.

No ascites or pleural effusion.

Other Findings:

An IUD is seen within the uterus.

Right ovarian active cyst (likely functional).

1) The rest of PET / CT study appears normal.

Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, and urinary tract.

CONCLUSION:

- PET-CT study for reported case of breast carcinoma showing:
 - ✓ Metabolically active (FDG avid) right breast bi-focal neoplastic soft tissue lesions; associated with FDG avid regional metastatic axillary LNs.
 - ✓ No FDG avid distant metastases.

Dr. Sherif Filfil.

Prof. Ahmed Wafaie MD, FRCR

Pat. Name: Basma Hafez Hussein	Pat. ID: 3627869
Procedure Name: 3D Digital Mamo with Tomo -Bo	Date: 20/08/2026

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHIC
EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY : right breast pathologically proven IDC grade III.
IMPRESSION :
LEFT BREAST: (BIRADS 1)
Normal study

RIGHT BREAST: (BIRADS 6)
UOQ, UIQ pathologically enhancing masses with ipsilateral pathological axillary lymph nodes.

DETAILED FINDINGS :

TECHNIQUE:

Craniocaudal & Mediolateral Oblique views
Contrast mammography/ ultrasound

Mild back ground parenchymal enhancement.

LEFT BREAST:

- o **LOW ENERGY (NON CONTRAST) IMAGES:** No dominant mass lesion, spiculations or suspicious microcalcifications.
- o **RECOMBINED (CONTRAST IMAGES):** showed no pathological uptake.
- o No signs of malignancy detected.
- o Normal skin/nipple/areolar complex.
- o Preserved axillary lymph nodes.

RIGHT BREAST:

- LOW ENERGY (NON CONTRAST) IMAGES: multiple ill defined spiculated masses. Cluster of microcalcifications at upper midline posterior third.
- RECOMBINED (CONTRAST IMAGES): multiple ill defined heterogeneously enhancing masses at the UOQ and to a lesser extent UIQ.
- **Distance from the nipple:2.1 cm.**
- **The largest measure 19x18mm.**
- The overall area measures: 9x10x8 x5 cm.
- Normal skin/nipple/areolar complex.
- Multiple level I (more than 3 L.N) pathological axillary lymph nodes the largest measure 19x5mm. .
- No chest wall invasion.

Thanks for your Reference,
Dr Hanaa Abo El Magd

Signature Dr hanaa
18/03/20129:29 PM